

30-Day and 1-year mortality in emergency general surgery laparotomies: an area o

Answer (EN)

Yes. The source paper frames emergency surgery as carrying poorer outcomes and higher mortality —citing recent studies reporting 30-day mortality of 14–15% —and explicitly states that its aim is to encourage prospective data collection, improvement of care, and strategies to establish best practice. This framing directly supports the view that 30-day and 1-year mortality following emergency laparotomy constitute an area of concern and a target for improvement.

中文速览

是。论文将急诊手术描述为结局更差、死亡率更高（引用近期研究 30 天死亡率 14–15%），并明确表明研究目的是促进前瞻性数据收集、改进照护并推动最佳实践策略的建立，因此支持 30 天及 1 年死亡率是需要关注和改进的领域。

Source evidence

“Emergency surgery is associated with poorer outcomes and higher mortality with recent studies suggesting the 30-day mortality to be 14-15%.” (arXiv:SciQ:ctx)

“We hope this will encourage prospective data collection, improvement of care and initiate strategies to establish best practice in this area.” (arXiv:SciQ:ctx)

Final decision: yes